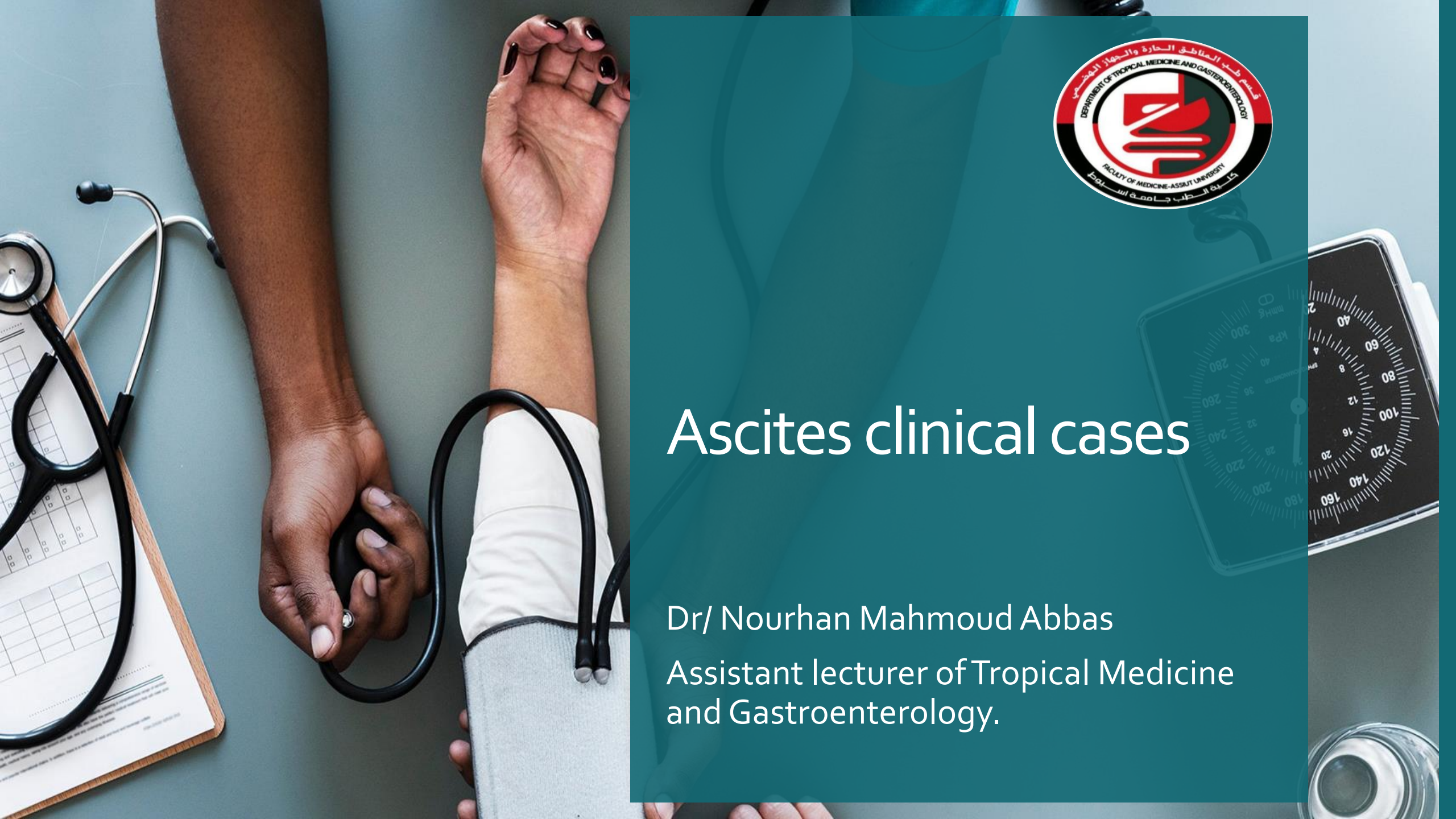


Ascites clinical cases

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Case 1

- A 65-year-old man goes to his general practitioner (GP) complaining of increasing abdominal distension and anorexia of 4 weeks duration followed by 2 weeks of jaundice. He denies any weight loss, changes in bowel habits nor vomiting.
- He takes no regular medications he has drunk beer for about 10 years, but stopped 10 days ago.
- On examination:

  He has 5 spider naevi on his upper chest wall.

  Abdominal examination: reveals 6 cm firm palpable rt lobe of liver and

  ascites is confirmed by shifting dullness
- Investigations show :

  raised bilirubin, thrombocytopenia, hypoalbuminemia.

Case 1

- **Diagnosis**

Decompensated liver chirrhosis complicated with ascites

- **Management**
- abd us to confirm diagnosis
- Ascitis fluid study
- Control of ascites:

salt free diet, diuretics, TIPS and up to liver transplant

Case 2

- A 55 years old male patient, presented to our Outpatient clinic with gradual increasing abdominal distention of 1 m duration associated with yellowish discoloration of sclera, dark colored urine and clay stool.

  Patient also had significant weight loss over the last month.
- On examination

Pt has jaundice, enlarged supraclavicular lymph nodes

Palpable gall bladder mass.

Ascites by shifting dullnes

Investigations

Hyperbilirubinemia mainly direct and hypoalbuminemia

Case 2

- **Diagnosis**

malignant ascites

- **Management**
- Abd us/MSCT : showing biliary system, confirm mass, exclude mets, confirm ascites and detect abdominal lymphadenopathy
- ERCP : to relieve biliary obstruction then surgical consultation

Case 3

- A 65 years old male patient, known to be HCV related liver cirrhosis presented to emergency room by gradual disturbed conscius level.

Patient also has history of fever, abdominal pain and distention of 3 days durations.

- on examination

Pt drowsy with flapping tremors
Jaundiced
Abdominal tenderness with ascites by shifting dullnes

- Investigations

Leucocytosis ,Thrombocytopenia, hyperbilirubinemia and hypoalbuminemia

Case 3

- Diagnosis

Pt mostly developed SBP which precipitate hepatic encephalopathy

- Management:

-Ascitic fluid study (PMN count)
-Ascitic fluid culture (bedside test)
-Start antibiotics (cefotaxime/ceftriaxone)
-Salt free albumin if indicated
-Prophylactic antibiotics on discharge

Case 4

- A 75 years old female patient, had HBV related liver cirrhosis presented to emergency room by gradual disturbed conscius level.
- She was on regular medications: lactoluse syp, aldactone 400 mg/day and lasix 40 mg twice/day .
- On examination

was hypotensive 70/40

Abdomen : dull flankes

No ll oedema

Case 4

Management:

Fluid resuscitation

Correct electrolyte imbalance

Anti encephalopathy measures

Adjust diuretics dose on discharge

Thank you

